DRUGS · VERTIGO AND MÉNIÈRE'S DISEASE

Betahistine

Betahistine is a histamine analogue prescribed widely for Ménière's disease to reduce attacks of vertigo. It is thought to improve cochlear and vestibular blood flow and to modulate vestibular nuclei activity. The best trial to date (BEMED, 2016) found no difference from placebo in attack frequency at either standard or high dose, so it should be offered honestly, as a low-risk option with uncertain benefit.

1 CHAPTER ONE At a glance

The drug on one page.

1.1 Summary

BETAHISTINE

	SUMMARY
Class	Histamine analogue: weak H1 agonist and potent H3 antagonist
How it works	Proposed: increased inner ear microcirculation via H1 vasodilatation and H3-mediated increase in histamine release; reduced firing of vestibular nuclei
ENT uses	Ménière's disease (reduction of vertigo attacks); widely used, without good evidence, for other vertigo
Adult dose	16 mg three times daily or 24 mg twice daily with food; some use higher doses (up to 48 mg three times daily) on specialist advice
Child dose	Not recommended under 18 years
Avoid	Phaeochromocytoma; caution in asthma, peptic ulcer disease and with antihistamines
Pregnancy	Avoid unless essential

2 CHAPTER TWO Using it in ENT

Indications.

2.1 Ménière's disease

The first steps are explanation, salt restriction, attention to caffeine and stress, vestibular rehabilitation, and acute vestibular sedatives for attacks. Betahistine may be offered as a preventive; review it at 3 to 6 months and stop it if attacks have not reduced. Treatment escalates to intratympanic steroids, then intratympanic gentamicin or surgery for disabling, frequent attacks.

2.2 Evidence

In the BEMED trial, neither 24 mg twice daily nor 48 mg three times daily reduced attacks more than placebo over 9 months; attacks fell in all groups, illustrating the natural fluctuation of the disease.

3 CHAPTER THREE Safety

What goes wrong.

3.1 Adverse effects

Nausea, dyspepsia, headache, and rarely rash or pruritus. It does not sedate.

3.2 Interactions

Antihistamines may in theory reduce its effect; MAO-B inhibitors raise its levels.

4 CHAPTER FOUR For the examination

What the examiner wants to hear.

4.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- The proposed mechanism.
- The **BEMED** trial result and how to counsel patients.
- Where betahistine fits in the stepwise management of Ménière's disease.

4.2 Common pitfalls

COMMON PITFALLS

- Prescribing betahistine indefinitely for any dizziness.
- Using it as an acute treatment for a vertigo attack.

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